

MODULE

4

Heart & Small Intestine

The Posterior Circuit Opens – Plus: The Five Shu Points

Third channel pair of the 12 Primary Meridians – first pair of the Posterior Circuit

This Module Covers:

- The Five Shu Points – finally addressed, straight from Dr. Zhang's own slides
- A formal definition of “crossing point”
- The Heart Meridian of Hand-Shaoyin (HT) – course, points, syndromes
- The Small Intestine Meridian of Hand-Taiyang (SI) – course, points, syndromes
- Ballroom & DanceSport relevance

Sourced from the 2026 Lecture 4 deck (Dr. Zhang, 41 slides) and the Week 4 class transcript.

This deck includes classical Chinese source text (Ling Shu citations) for each channel, not seen in Weeks 1–3 – noted where it adds something the English paraphrase doesn't.

How to Use This Module

HT and SI are the fifth and sixth of the 12 primary meridians — the first pair of the Posterior Circuit (Module 1, Section 6: HT to SI to BL to KI). This module also finally addresses the Five Shu Points, flagged as pending back in Module 1 — Dr. Zhang introduces them here, organically, as part of the Heart Meridian's running course, rather than in a separate special-points lecture.

1. The Five Shu Points

Slide 19 of this lecture introduces these directly, described as new content students had not previously covered. Each of the 12 primary meridians has five specific points below the elbow or knee, named for how far along the channel's flow of Qi has progressed at that point — from where Qi 'emerges' at the fingertip/toenail to where it 'enters' near the elbow/knee.

Point	Meaning	Location	Clinical Application
Jing-Well	"Where it emerges"	Tips of fingers and toes	First aid, clearing heat, relieving pain, reducing inflammation
Ying-Spring	"Where it flows"	Before the metacarpophalangeal/metatarsophalangeal joints	Feverish diseases, heat-related disorders
Shu-Stream	"Where it pours"	After the metacarpophalangeal/metatarsophalangeal joints	Heaviness in the body, joint pain, pain syndromes
Jing-River	"Where it travels"	Forearms or lower legs	Externally contracted diseases (colds, flu), cough, asthma
He-Sea	"Where it enters"	Near the elbow or knee joints	Disorders of the six Fu organs

Module 1 flagged meeting-point logic (Yuan-Source, Luo-Connecting, Five Shu, etc.) as deferred to a future Special Points module. The Five Shu Points specifically are now covered — the rest (Yuan-Source, Luo-Connecting, Xi-Cleft, Eight Confluent) remain open, noted again in this module's Still Open section.

Crossing Point — A Formal Definition

Also given directly in this lecture, worth having on record precisely: "A crossing point is defined as a specific acupoint situated at the convergence or crossing of two or more meridians." Every crossing point named in Modules 1–3 fits this definition exactly — this is simply the first time the lecture states it as a formal rule.

2. The Heart Meridian of Hand-Shaoyin (HT)

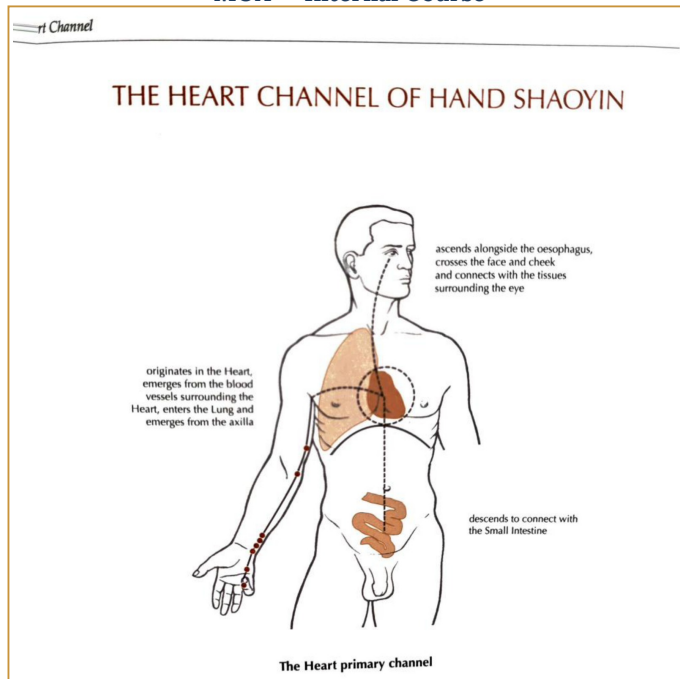
Meridian Clock: 11:00 a.m.–1:00 p.m. Part of the Posterior Circuit, paired as Zang with the Small Intestine (Fu). The Posterior Circuit's first channel, opening where Module 3's Spleen Meridian handed off.

Running Course — Three Branches

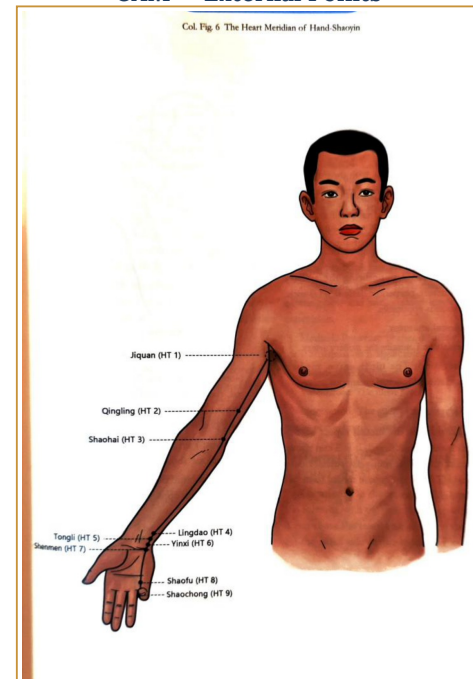
1. Internal descending portion: originates in the heart, emerges to spread over the "heart system" (the tissues connecting the heart to other Zang-Fu organs), passes through the diaphragm, and connects with the small intestine (its connecting organ).
2. The branch (ascending portion): from the heart system, runs alongside the esophagus to connect with the "eye system" (the tissues connecting the eyes with the brain). The transcript notes a classical line worth keeping: "the eye is the window of the heart" — both the Heart and Small Intestine Meridians connect with the eye.
3. The branch (straight portion): from the heart system, goes upward to the lung, then turns downward and emerges from the axilla at Jiquan (HT1). Runs the posterior border of the medial upper arm — behind the Lung Meridian and the Pericardium Meridian — down to the cubital fossa (HT3), the medial-posterior forearm (HT4, HT5), the pisiform region (HT6, HT7), the palm (HT8), ending at the tip of the little finger, radial side (Shaochong, HT9).

Clinical emphasis from the Week 4 transcript, not on the slides themselves: Jiquan (HT1) is specifically called out as important for treating stroke. Shaochong (HT9), the Jing-Well point, is emphasized for emergency presentations – severe chest pain, difficulty breathing, palpitations – consistent with the Jing-Well category's general first-aid function from Section 1 above.

MOA – Internal Course



CAM – External Points



Left: internal course showing the Heart/Small Intestine/Lung/eye connections (MOA). Right: the 9 external points, Jiquan (HT1) to Shaochong (HT9) (CAM).

First and Last Points

	Point	Category
First point	Jiquan (HT1)	—
Last point	Shaochong (HT9)	Jing-Well Point

Summary of the Heart Meridian

- Pertaining organ: Heart. Connecting organ: Small Intestine.
- Running course: chest to hand, along the posterior-medial arm.
- Total points: 9 – the smallest channel covered so far.

Main Syndromes & Indications

A. External course symptoms

- General feverishness, headache.
- Pain in the eyes, pain along the back of the upper arm.
- Dry throat, thirst.
- Hot or painful palms; coldness in the palms and soles of the feet.
- Pain along the scapula and/or medial aspect of the forearm.

B. Internal organ (Heart) symptoms

- Blood pressure changes, palpitation.
- Pain or fullness in the chest and ribs, or below the ribs.
- Mental disorders – anxiety, depression, poor memory.

The transcript is explicit about why the internal symptom list spans both cardiovascular and mental/emotional territory: the Heart has two main functions in this framework — governing Blood and the blood vessels, and “housing the mind” (Shen). The second function is why Heart Meridian points, especially those near the wrist, are used clinically for insomnia, poor memory, depression, and anxiety, not only for cardiovascular symptoms.

Ballroom/Dance Relevance — the axilla line and performance-anxiety physiology (original synthesis, not lecture content)

HT's straight portion runs the posterior-medial arm from the axilla — directly under sustained load in any extended-arm frame position, and the exact territory that tightens under prolonged overhead or extended-arm holds. The dual Heart function the transcript describes — blood vessels and Shen/mind together — also maps onto something dancers know from experience even without the TCM framing: physical cardiovascular strain and performance anxiety show up through the same channel, not as two separate problems.

3. The Small Intestine Meridian of Hand-Taiyang (SI)

Meridian Clock: 1:00–3:00 p.m. Part of the Posterior Circuit, paired as Fu with the Heart (Zang).

Running Course — Three Branches

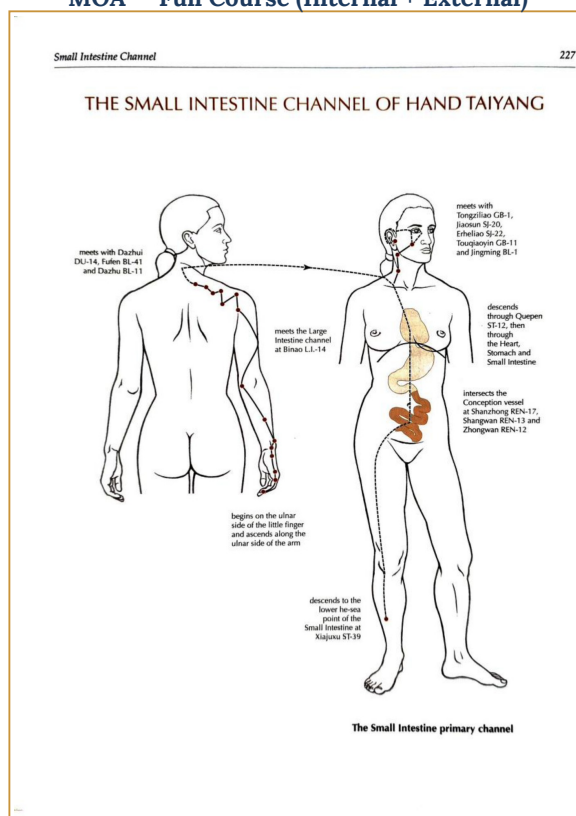
1. The main course: starts at the ulnar side of the tip of the little finger (Shaoze, SI1). Follows the ulnar side of the dorsum of the hand to the wrist, emerging at the styloid process of the ulna (SI4, SI5). Ascends the posterior forearm (SI6, SI7), passes between the olecranon of the ulna and the medial epicondyle of the humerus (SI8), runs the posterior-lateral upper arm to the shoulder joint (SI9, SI10), circles the scapular region (SI11–SI13), meets Dazhui (GV14) at the top of the shoulder, turns down to the supraclavicular fossa, connects with the heart, descends alongside the esophagus, passes the diaphragm, reaches the stomach, and finally enters the small intestine, its pertaining organ.
2. The branch (from the supraclavicular fossa): ascends the neck to the cheek, to the outer canthus (Tongziliao, GB1), entering the ear at Tinggong (SI19).
3. The branch (from the neck): rises to the infraorbital region (Quanliao, SI18), continues to the lateral side of the nose, and reaches the inner canthus (Jingming, BL1), linking with the Bladder Meridian of Foot-Taiyang — the next channel in the Posterior Circuit.

Crossing Points

This channel has an unusually well-documented crossing-point list — the source gives precise locations, not just names, for each:

Point	Location
Zhongwan (CV12)	Anterior midline, 4 cun above the umbilicus
Shangwan (CV13)	Anterior midline, 5 cun above the umbilicus
Jingming (BL1)	Slightly above the inner canthus
Dazhu (BL11)	1.5 cun lateral to Taodao (GV13), at the lower border of the T1 spinous process
Fufen (BL41)	3 cun lateral to the Governor Vessel, at the lower border of the T2 spinous process
Dazhui (GV14)	Below the C7 spinous process, roughly level with the shoulders
Tongziliao (GB1)	0.5 cun lateral to the outer canthus
Erheliao (SJ22)	Anterior/superior to Ermen (SJ21), level with the root of the auricle

MOA — Full Course (Internal + External)



From MOA – back and front views with every crossing point labeled directly on the figure. No CAM figure is used here: the CAM reference PDF's channel sequence skips from Fig. 6 (Heart) to Fig. 8 (Bladder), and Fig. 7 (Small Intestine) is genuinely absent from the source – not a page this module failed to find.

First and Last Points

	Point	Category
First point	Shaoze (SI1)	Jing-Well Point
Last point	Tinggong (SI19)	—

Summary of the Small Intestine Meridian

- Pertaining organ: Small Intestine. Connecting organ: Heart.
- Running course: hand to head, along the posterior-lateral arm, up through the neck and cheek.
- Total points: 19.

Main Syndromes & Indications

A. External course symptoms

- Numbness of the mouth and tongue.
- Pain in the neck or cheek; sore throat; stiff neck.
- Pain around the lateral aspect of the shoulder and upper arm.

B. Internal organ (Small Intestine) symptoms

- Pain and distention in the lower abdomen, sometimes extending to the waist.
- Diarrhea, or abdominal pain with dry stool or constipation.

Ballroom/Dance Relevance – the shoulder-circling course and neck tension (original synthesis, not lecture content)

SI's main course literally circles the scapular region before crossing the top of the shoulder – almost exactly the pattern of tension that builds from sustained frame work and repeated overhead arm positions. 'Stiff neck' and shoulder/upper-arm pain along the channel are worth knowing as SI territory specifically, distinct from LI's shoulder symptoms back in Module 2: LI runs the front/lateral line into the face, SI runs the back of the shoulder into the ear – different complaint, different channel.

4. Ballroom & DanceSport Relevance – Summary

Original synthesis, not lecture content – flagged as such throughout, consistent with Modules 1–3.

- HT and SI open the Posterior Circuit's dance-relevant territory: HT covers the inner-arm/axilla line and the blood-vessel/Shen dual function; SI covers the shoulder-circling line into the neck and ear.
- Between LI (Module 2) and SI (this module), the shoulder is now covered from two distinct channel angles – front/lateral (LI) and back/circling (SI) – which is likely to matter for the capstone's injury-pattern framework when shoulder complaints don't cleanly separate by front vs. back.

5. Still Open / Not Yet Sourced

- Yuan-Source, Luo-Connecting, Xi-Cleft, and Eight Confluent Points – the Five Shu Points are now covered (Section 1), but these other meeting-point categories remain deferred to a future Special Points module.
- Finger measurement's specific cun ratios – still unconfirmed, carried over from Module 2.
- The exact abdominal/chest channel spacing relative to the midline – still unconfirmed, carried over from Module 3.

Source Notes

Primary source: 2026AC300Lecture_4vivian_1.pdf (41 slides, Dr. Zhang, Lecture 4) and AC300_Week_4_Transcript.txt (class transcript). This is the first module to draw on a '2026'-series deck rather than an older 'Week_N' deck – per the project's established source hierarchy, the 2026 decks are authoritative where both exist. MOA figures are the same reference figures used across the weekly study kits; the CAM reference PDF is missing its Small Intestine figure (Section 3 explains this directly rather than substituting silently).